

auracare

Revolutionizing The Care Cycle

THE PROBLEM

Personalized medicine needs the full picture, that infrastructure doesn't exist.

SHORT CONSULTATIONS

5 Minutes

The average duration of patient consultations in more than half of the world.¹

DATA DEFICIT

44%

The percentage of consultations where a lack of patient data harms the final diagnosis.²

FRAGMENTED NARRATIVE

Daily life

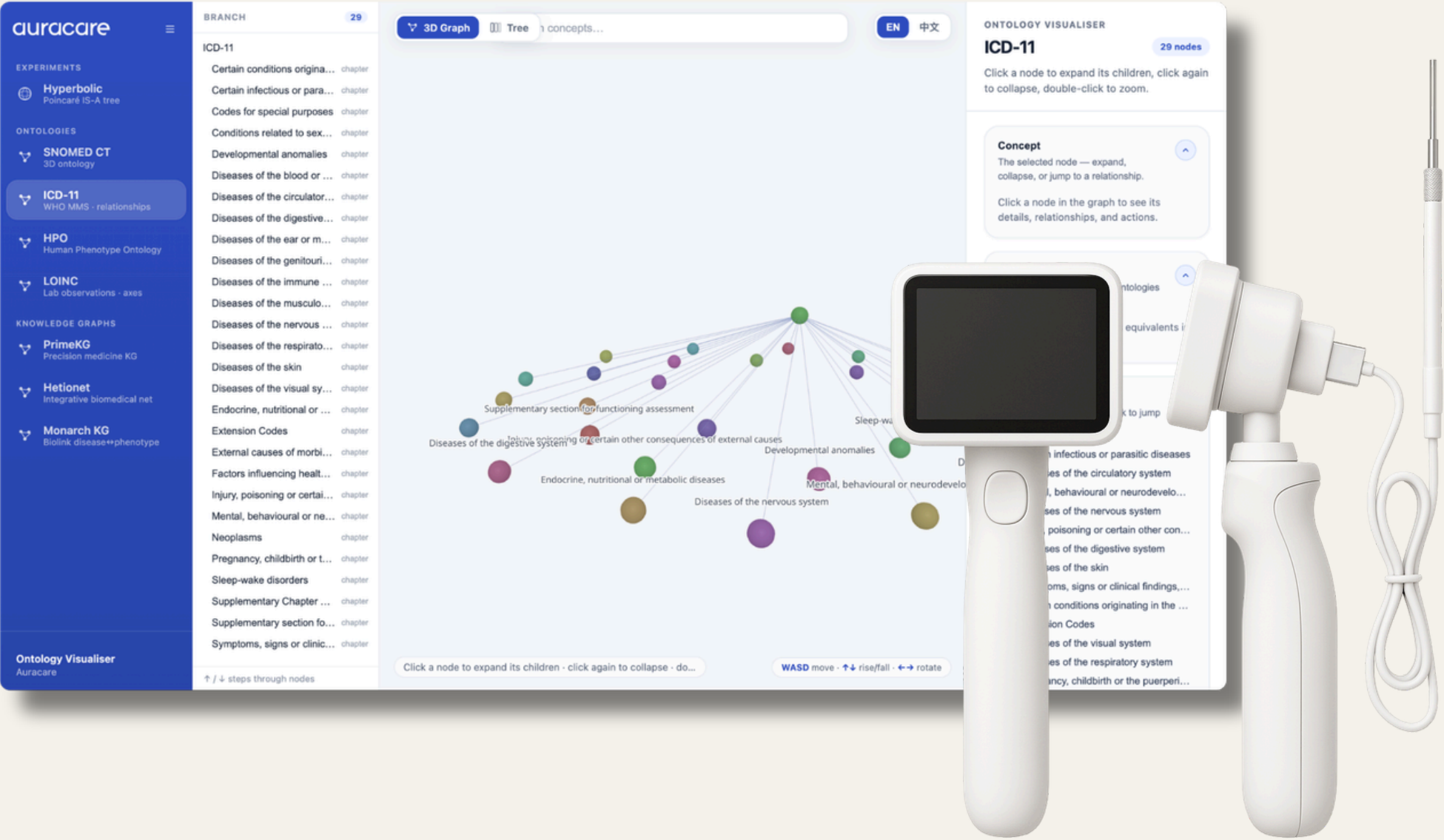
The EHR is a series of snapshots: long gaps with no context in between.

Consultations are just a snippet. Data is incomplete. The full picture is obscured.

MEET AURACARE

A full support system for patient care.

Auracare brings a patient's daily life, history, tests and the live consultation into one coded record, then reasons across this data to support the clinician through diagnosis, treatment and documentation.



STRUCTURE

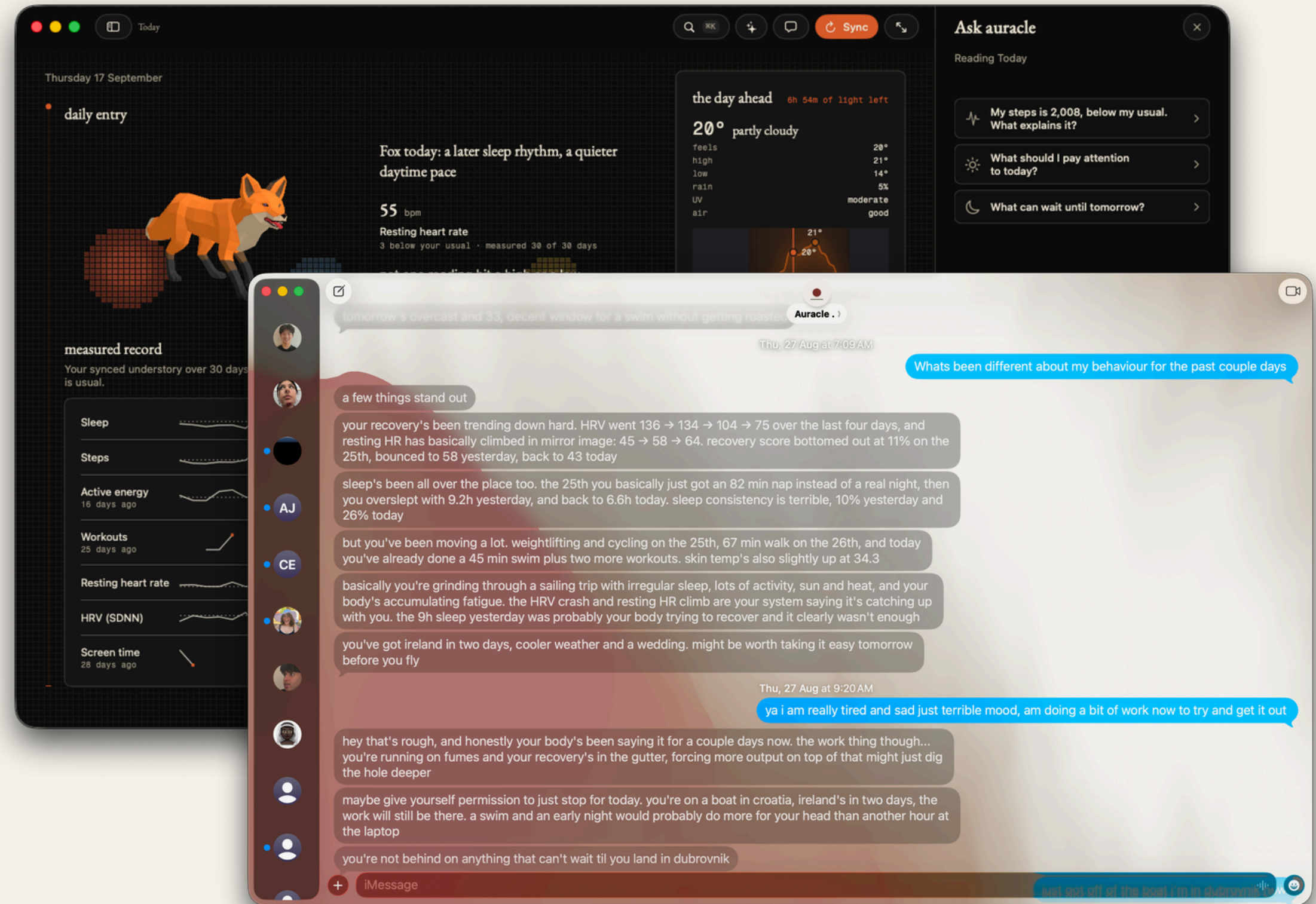
The Core Offering

DATA AGGREGATION ENGINE Encodes daily life A patient portal that aggregates user data and encodes clinical findings using SNOMED CT standards.	DETERMINISTIC INFERENCE CORE Reasons with the data Our data engine analyzes patient data and cross-references it with established clinical guidelines to propose ranked differential diagnoses for the clinician.	INTAKE KIT Captures the basics Blood pressure, temperature, height and weight measured by proprietary hardware, sent straight into the CDSS with no manual entry.
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THE WEDGE • STAGE 1

We're starting at the patient.

Stage 1 is Auracare's **data aggregation engine**, shipped as a patient-oriented portal for clinics to manage customer context and history pre-consult. It learns a person's daily life from existing online connections and external information, encoding everything into SNOMED CT, fed directly to our CDSS.



THE ENTRY POINT

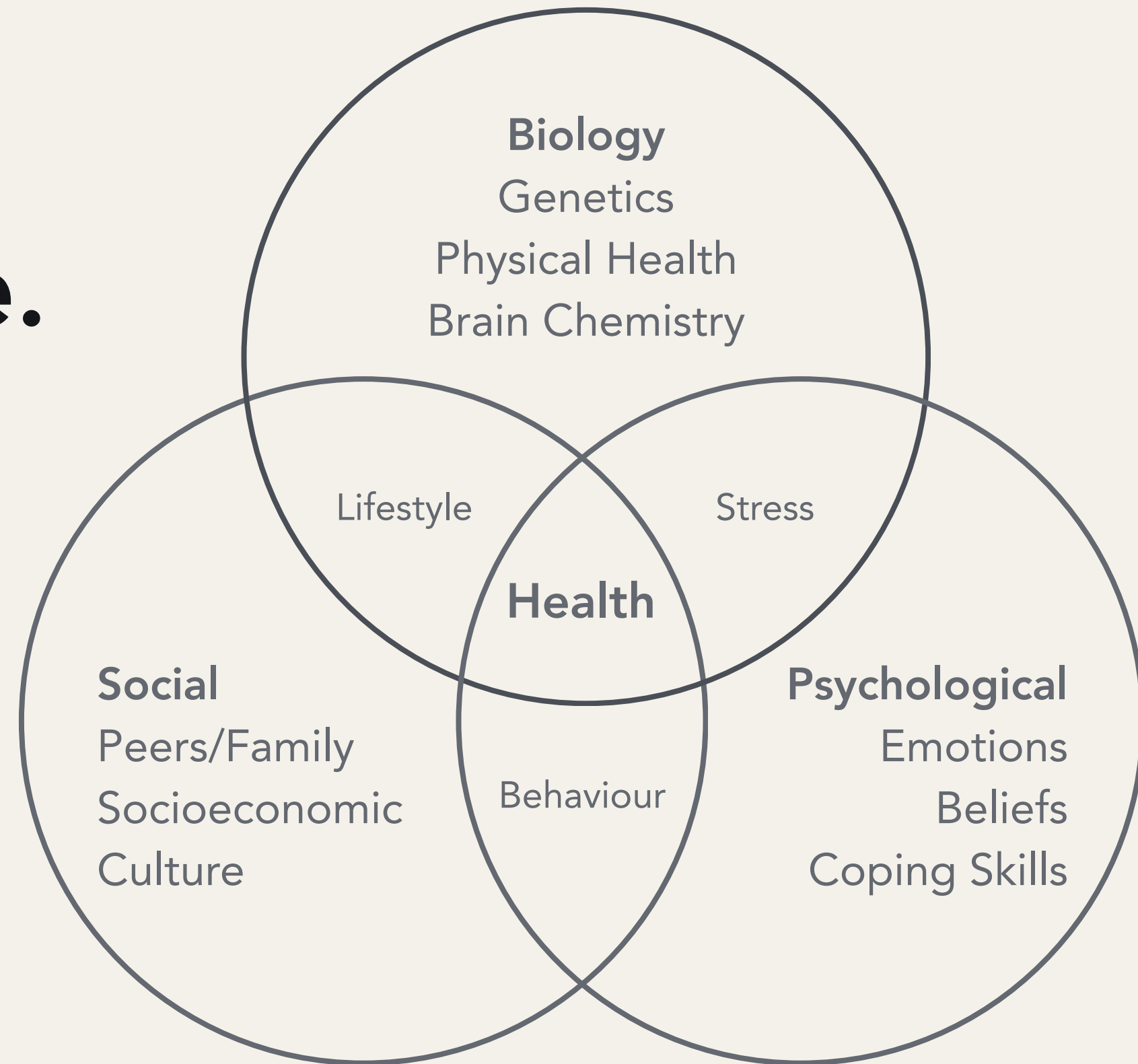
Consultations don't paint the full picture.

BIOPSYCHOSOCIAL MODEL

90%

of a person's health is influenced by non-medical factors, excluding genetics.³

Our Data Aggregation Engine completes the patient history.



THE STAGES

CDSS: The Full Picture

STAGE 1

Connectors

A full lifestyle picture shared by the patient, always up to date.



Data Aggregation Engine

Collating patient data from all sources:
The complete picture.



Third Party Information

Prescriptions, consultations, and imaging from a patient's past check-ups.

STAGE 2

Live Consultation

Recorded and transcribed with detail and encoded.



Deterministic Inference Core

Reasons against clinical guidelines to suggest a personalized care plan.



Proprietary Hardware

Blood pressure, temperature, height and weight, coded on entry from our own hardware.

ENCODING
.....
SNOMED CT · ICD-11
LOINC · HPO

STAGE 3

Integrated Documentation

Documentation standardized to the hospital's existing systems.

Ranked differentials

Ordered by likelihood and evaluated against the patient's own history.

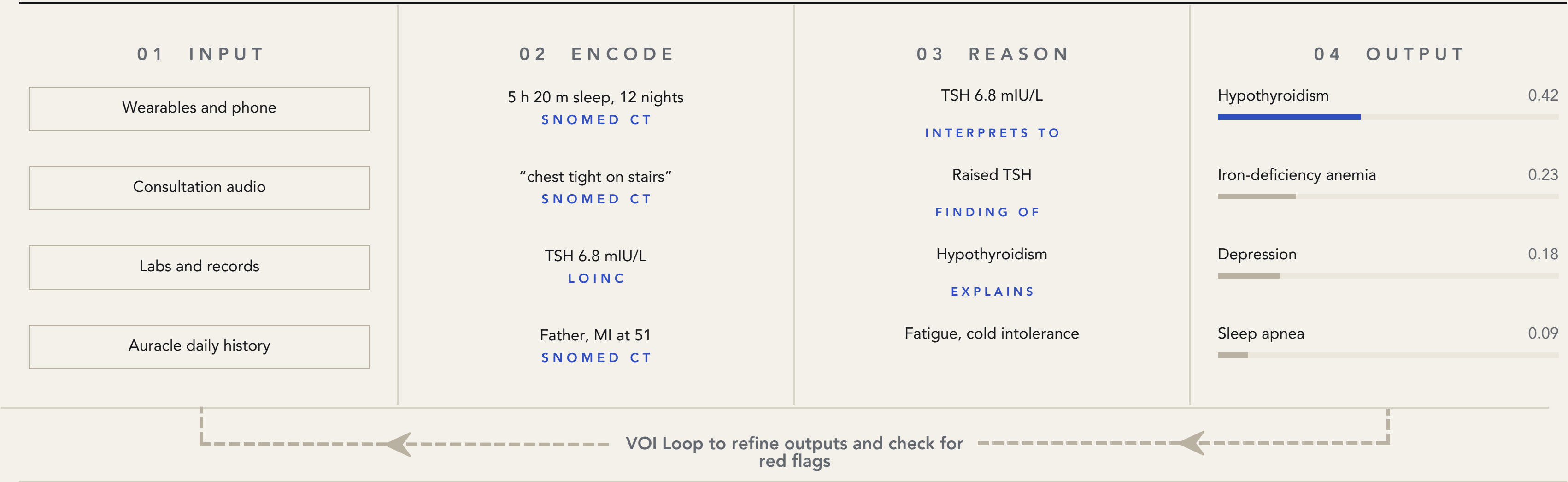
Supply Optimization

Reducing stock / workforce wastage and tailoring care selection.

Glass box, not a Black Box

Full data transparency over our reasoning model.

Data Aggregation Engine → Deterministic Inference Core → Outputs; all within a feedback loop.



OUR OWN HARDWARE

We build the hardware, so the data comes easy.

A full clinical decision needs measurement.

We manufacture our own examination devices through OEM partnerships in China,
so vitals arrive normalized and coded to our need without relying on third-party hardware. These are then
coded automatically allowing easy tracking over time.

Blood pressure monitor Clinical-grade readings, sent straight into the CDSS.	Thermometer Temperature taken at intake, coded on entry.	Scale and height measure Weight and height recorded automatically, appended to their data record.
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Intake kit connected to the CDSS in early 2027.

INITIAL TARGET CUSTOMER

The Entry Point

Private outpatient healthcare conglomerates that own and run networks of outpatient clinics; brands and specialties under one umbrella entity that overlooks ownership and operations.

Our beachhead is East Asia, starting in Hong Kong, with clinical trials at EC Healthcare and Hyzen Hospital, and the Chinese Academy of Sciences as a research and product development partner.

More patients per clinician THROUGHPUT

- 1 The record writes itself as the consultation happens. Time returned per visit leads to more patients per hour.

One patient across every clinic REFERRALS

- 2 A diagnosis today is on the record as whatever word that doctor chose that day, so nothing transfers between clinics. Coded records make internal referral work.

A structure to build on AUTOMATION

- 3 Nothing can be automated on notes that vary by author. Encoding is the precondition for stock optimization, audit and every automatised tool introduced.

MONETIZATION

Direct to Digital

API access for clinics; generalized features with basic support, no tailored integration into existing systems

Direct to Service

Sold to the groups that own the clinics, priced on the costs reduced through decreased wastage (both clinician time and stock)

TRACTION

Building Momentum

Funding, partnerships and clinical trials lined up.

UK PHARMACY PARTNERSHIPS, CDSS PILOT	28
UK GOVERNMENT FUNDING AWARDED	\$134k
ANGEL INVESTMENT, HONG KONG	\$400k
CLINICAL TRIAL, EC HEALTHCARE	Nov 2026
CLINICAL TRIAL, HYZEN HOSPITAL	Feb 2027
REASEARCH PARTNERSHIP, CHINESE ACADEMY OF SCIENCES (ZIAT)	Since 2025

Completed **Founders Factory Healthcare: Hospital to Community** cohort and secured a **3-year Vanta contract** covering 5 compliance frameworks.

THE JOURNEY

The Road to the Clinic



REGULATION

A clear path through regulation.

Doctors are still making the decisions. We make sure the data behind it is accurate, traceable and kept private.

HONG KONG

Class II

medical device. Registration is **voluntary** today, so we can work with clinics now. Full listing targeted 2028.

CHINA & FDA

Unregulated

The nature of the product makes it not classified as a medical device in both China and the USA (FDA). No restrictions or mandatory wait time to trial and sell in China and countries following FDA guidelines.

OPPORTUNITY

A Large Clinical Market

\$30B

Total addressable market⁴

WHO Workforce: 12.7 million medical doctors worldwide × \$2,400 per clinician per year.

\$11B

Serviceable obtainable market

The segments our launch markets and clinical route actually reach.

COMPARABLES

OpenEvidence
\$12B valuation

Bayesian Health
~\$40m valuation

THE TEAM

Clinical, engineering and manufacturing



Tanush Pandey

CO-FOUNDER · CEO & CMO

Medicine at Imperial College London. Leads clinical strategy, safety, and research, along with HCP relations.



Stephen Okita

CO-FOUNDER · CTO

Computer science at UC Berkeley. Sets the architecture and product direction; business relations in San Francisco.



Hinlun Chen

CO-FOUNDER · COO

PPE at LSE; runs an OEM with two factories. Leads operations and partnerships in Hong Kong and China.



Julius Chu

CPO

Computer science & Entrepreneurship at NYU. Leads product, design system, the consumer experience.



Karam Elradie

HEAD OF ML

Leads machine learning (PhD): the neuro-symbolic core and graph models behind our reasoning.



UC Berkeley



auracle



THE ASK

We are raising a seed round

\$2.65M

12-month runway

\$2.41M deployed, plus a ~10% contingency.

Team	8 personnel, \$50–200k per head	\$910k
Clinical & regulatory	Clinical trials, quality system and international listing, intake kit	\$800k
Operating & G&A	IP and patents, legal, insurance, admin, travel	\$300k
Compute & data	AI training runs, processors and data	\$400k
Contingency	External, circumstantial costs spread across other expenses	\$240k
TOTAL DEPLOYED		\$2.65M

auracare

GET IN TOUCH

Be a pioneer.

We are raising a seed round to build the core engine,
run our clinical trials, and take Auracare to clinics.

Tanush Pandey

CEO & CMO · EUROPE

tanush@auracare.org.uk

Stephen Okita

CTO · NORTH AMERICA

stephen@auracare.org.uk

Hinlun Chen

COO · ASIA-PACIFIC

hinlun@auracare.org.uk

Sources

- 1 Irving G, et al. International variations in primary care physician consultation time: a systematic review of 67 countries. BMJ Open 2017;7:e017902.
- 2 Smith PC, et al. Missing Clinical Information During Primary Care Visits. JAMA 2005;293(5):565-571.
- 3 Hayes TO, Delk R. Understanding the Social Determinants of Health. American Action Forum 2018.
- 4 Boniol M, et al. The global health workforce stock and distribution in 2020 and 2030. BMJ Global Health 2022;7:e009316.

Market figures from the OpenEvidence and Bayesian Health valuations are drawn from published funding rounds and press coverage.
